



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نجلاء سيد على حسين

العنوان : ممممممممم

الحي / المدينة : العياط

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 27806062100326 رقم التليفون : 01154811667

العمر (تاريخ الميلاد) : 06-06-1978 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نجلاء سيد علي حسين National ID: 27806062100326	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
وزارة الداخلية

تجاء
سيد علي حسن عمار سليم

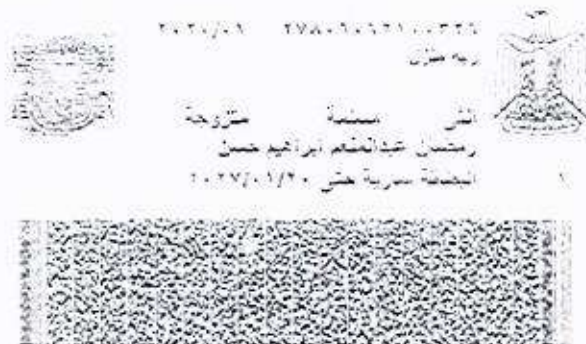
من ٢٢ يونيو
مركز الضيافة - الجيزة

١٩٧٨/٠١/٠١

٢٧٨ ٠٦ ٠٦ ٣١ ٠٠٣ ٢٦

٢٢٩٧١٥٨٦

B7



Pathology Report

Name: اسماء محمد عتیق Age: 42 years.

Clinical data: Left breast cancer, Margins and sentinel L.Ns assessed by F.S.

Specimen type: 1) WLE, 2) Sentinel lymph nodes.

Date (Received): 12/4/2021

Date (reported): 25/4/2021

Lab. Number: 786/21

Hospital Number: 439/21

Gross Description:

Four specimens were received and designated as:

- 1) A fibro fatty tissue 7x4x3cm with a covering skin ellipse 4x2cm, cut section revealed a greyish white gritty mass 1.5x1x1cm, Reaching the deep margin grossly
- Another greyish white mass near the lateral margin measure 0.5x0.5x0.5 cm away from the 1st one by 1cm. Margins are superior 0.5cm, inferior 2.5cm medial 2.5cm and lateral 1cm.
- 2) 5 L.N greyish with fatty tissue 7x5cm entangling 6 L.N 0.3cm up to 1.5cm in diameter left breast mass tissue.
- 3) Inferior margin 5x4x1cm fatty tissue.
- 4) Deep margin 5.5x1cm fatty tissue.

Microscopic Examination:

- 1) Both grossly described masses show high grade DCIS with focal stromal invasion (2mm in largest diameter).
Dissected and extra margins are free of invasive tumor, however lateral margin is close (1mm) to a focus of DCIS.
- 2) Sentinel lymph nodes negative for deposits (0/6).

Diagnosis:

Left breast mass, Wide local excision and sentinel lymph nodes:
Invasive duct carcinoma with major DCIS, T1 N0 Mx. Margins are close (lateral).
Lymph nodes negative (0/6).

Prof. Dr. In-an Gonda.

Name: naglaa sayed

Date: 5/11/2021

BILATERAL DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND:

45 yrs old female with left breast lump.

Findings:

Left breast shows architecture distortion retro-areolar
right breast shows hyperechoic lesion with indistinct margin overall
decreased density ACR B.

Complementary ultrasound left breast showed a corresponding
hypoechoic mass at 3 o'clock retroareolar position measuring about 2.1
x 1.6 cm with irregular margin and posterior anechoic shadowing.

Right breast shows small irregular lesion retro-areolar at 12 O clock with
spiculated margin measured about 5x4mm.

Associated pathologically bilateral axillary nodes.

Conclusion:

Bilateral suspicious lesion as described above for biopsy correlation BI-

RADS 4 b

Dr. Marwa abdelhakim MSc

Women's Health Outreach Program

Name: NAGLAA SEID ALI

Date: 7/3/2021

Contrast Enhanced Spectral Mammography:

Clinical summary:

Left breast proved malignancy DCIS with microinvasion, ER-ve, PR weak +,
HER 2 over expression and Ki 67 20-25%.

Findings:

Moderate background enhancement.

After contrast administration.

- 1) Left upper midline middle third (index lesion) moderate heterogeneous irregular mass enhancing lesion with spiculated margins, about 3.5 cm from the nipple, measures about 2 x 3 x 3.5 cm.
- 2) Left nodular non mass enhancement is seen at the UIQ about 5.7 cm from nipple, corresponding to the 9:00 lesions. Measures 8 x 5 and 4 x 2 mm? Benign.
- 3) Left enlarged axillary lymph node with cortical thickness 4 mm.
- 4) Right 12:00 (retroareolar) mass enhancement measure 7 x 3.4 mm? benign.
- 5) Right LIQ homogeneously enhancing mass, located in the middle third measures 1.5 x 1.5 cm? Benign.

Conclusion: (BIRADS 6= pathologically proved)

- Left unifocal upper midline malignant mass.
- Left indeterminate axillary L.N (biopsy was performed).

Women's Health Outreach Program

Name: Naglaa Seid Ali

Date: 14-3-2021

Bilateral High Resolution Ultrasonography

Clinical Presentation:

- Left pathologically proven DCIS (BIRADS 6)
- Right pathologically proven fibroadenoma (BIRADS 3)

Findings

Left breast:

- The left breast shows large ill-defined spiculated hypoechoic mass lesion located at 12 o'clock position measuring about 30.4 x 16.2 mm (Site of previous biopsy) with ductal extension measuring about 27 mm long.
- Another at least seven multiple ill-defined hypoechoic focal lesions located at the following sites:
 - o Four located at 12-1° clock position measuring about 7.5 x 4.7 mm, 4.1 x 2.5 mm, 3.7 x 2.4 mm and 5 x 2.7 mm
 - o One located at 2° clock position, measuring about 8 x 4.3 mm
 - o Two located at 9° clock position, measuring about 3.3 x 5.5 mm and 4.3 x 2.3 mm.
- Dilated retroareolar ducts measuring about 4.1 mm
- Enlarged axillary lymph nodes with focal cortical thickening reaching up to 4.2 mm.

Right breast:

- The right breast shows three well defined hypoechoic mass lesions at following sites:
 - o One located at retroareolar 12° clock position measuring about 7 x 3.4 mm
 - o Two located at 10° clock position, measuring about 7.5 x 4.4 mm and 5.2 x 3.4 mm.
- Dilated retroareolar ducts measuring about 4.1 mm
- Enlarged axillary lymph nodes with preserved oval shaped and central echogenic hila.

Conclusion:

NASSER INSTITUTE
ONCOLOGY CENTER



مركز
السرطان

NUCLEAR MEDICINE UNIT

Date: 10/7/2021

Patient Name: Nagla Said Ali

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- Degenerative changes at the dorsal and lumbar vertebrae.
- Otherwise the rest of surveyed skeleton is free from serious bone lesions.
- Both kidneys are normally seen.

Conclusion:

No definite evidence of metastatic bone deposits.
Follow up bone scan is advised.

Thanks for reference

Nuclear Medicine Unit
Huda Fathy, M.D.
Magdy Koth, M.D.



POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Known case of breast cancer.

1. Chest:

- Left breast irregular mass is seen measuring about 1.3 x 1.6 cm.
- Left lower lobe calcified nodule (3 mm) otherwise no sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Left ovarian cyst is noted measuring about 2.6 x 1.8 cm.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.
- Apart from dorsal osseous hemangiomas no lytic or sclerotic bone lesions.

OPINION:

- **Known case of breast cancer showing left breast mass as described above.**
- **No pulmonary or hepatic deposits.**
- **Left ovarian cyst for follow-up.**

Dr. Ahmed. M. Rozeka, MSc

Dr. J.A. Gomes, M.D.



Immunohistochemistry report

Report Number: m243

Female

النوع

جلد سميد على

التاريخ

التاريخ

42 Y

14/01/2021 09:58

تاريخ استلام العينة

20/01/2021 12:05

تاريخ التقرير

مركز صحة المرأة

مركز صحة المرأة

Primary Sample

Wax Block

Test

R. APR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu: CAP scoring system)

Comment

Technique

Sample received was treated by - CONFIRM anti-estrogen (ER) (SP4) Rabbit monoclonal primary antibody - OX188 anti- progesterone receptor (PR) (1E2) Rabbit monoclonal primary antibody.

Sample received was treated by - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Biotin-Labelled Ventana benchmark GX.

Sample received was treated by - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result

R. No nuclear staining of tumour cells (core biopsy)

P. weak nuclear staining of 5 % of tumour cells (core biopsy)

HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumour cells (core biopsy)

Ki67 : positive nuclear staining of 20-25 % of tumour cells (core biopsy)

Referral Lab

Comment

Conclusion

R. NEGATIVE

P. POSITIVE (SCORE 3/8)

HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)

Ki67 : THE PROLIFERATION INDEX OF THE TUMOUR IS 20-25 %

Pathologist

د. فريدلاند

Lab Director

د. فريدلاند

(Signature)



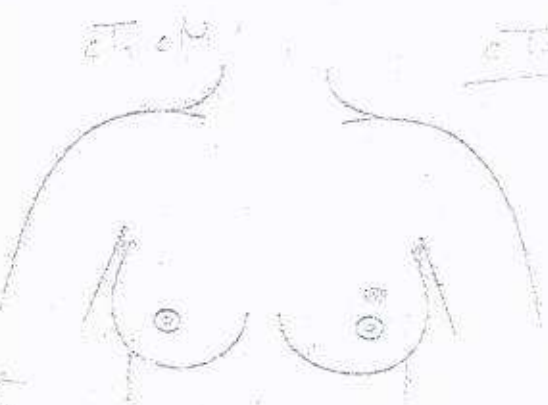
وزارة الصحة والشؤون الاجتماعية
مركز أورام دار السلام
فهرس سابقا

Name : Chen Jia
Age : 35
No. : 4/3/2021
Case (A)

MDT FORM

Clinical data :

U
27x16cm
R: R23
L: L23
neous



PR 3/3
1/2 1/3
10-67 = 6-65

slight retraction of the nipple
tubular IHC in the tumor
in some lymphatics

Reason of discussion:

MDT Outcome and Plan

MDT outcome: breast conserving surgery
= MDT

Plan: breast conserving surgery
with axillary lymph node dissection